

Procedure Date:  
Procedure Physician:  
Attending Physician/Copies To:

| Criteria                                   | Yes | No |
|--------------------------------------------|-----|----|
| Diagnostic Discrepancy                     |     |    |
| Primary Tumor Site Discrepancy             |     |    |
| HIPAA discrepancy                          |     |    |
| Prior Malignancy History                   |     |    |
| Donor/Synchronous Primary Noted            |     |    |
| Case is (circle): QUALIFIED / DISQUALIFIED |     |    |
| Reviewer Initials                          |     |    |

Carcinoma, infiltrating ductal, NOS 8500/3  
Site: Breast, NOS C50.9

1/21/11  
lw

UUID:982253AB-2C41-4E7A-9178-B690FFD9FF78  
TCGA-BH-A1ES-01A-PR



# PATIENT HISTORY:

1 & 2 TAKEN BY PATH DATE OF LMP  
DATE OF LAST DELIVERY:

PRE-OP DIAGNOSIS: R BREAST CANCER

POST-OP DIAGNOSIS: SAME

OPERATIVE PROCEDURE: R MASTECTOMY, SENTINEL NODE BX'S

CLINICAL HISTORY: 1 & 2 TAKEN BY PATH

MATERIAL SUBMITTED: A) RIGHT SENTINEL LYMPH NODE, PROCUREMENT BY SURGICAL PROCEDURE

# 1

B) RIGHT SENTINEL LYMPH NODE, PROCUREMENT BY SURGICAL PROCEDURE

# 2

C) RIGHT AXILLARY LYMPH NODE(S), PROCUREMENT BY SURGICAL PROCEDURE

D) RIGHT BREAST BIOPSY/MASS/SEGMENTAL MASTECTOMY, PROCUREMENT BY SURGICAL PROCEDURE

E) RIGHT SENTINEL LYMPH NODE, PROCUREMENT BY SURGICAL PROCEDURE

# 3

F) RIGHT SENTINEL LYMPH NODE, PROCUREMENT BY SURGICAL PROCEDURE

# 4

## INTRAOPERATIVE CONSULTATION:

FROZEN SECTION: 1) Sentinel node #1: 0.6 cm lymph node.

Frozen Section diagnosis: Negative for tumor by Touch Prep and Frozen Section.

2) Sentinel node #2: 0.8 cm lymph node.

Frozen Section diagnosis: Negative for tumor by Touch Prep and Frozen Section.

3) Sentinel node #3: 0.8 cm lymph node.

Frozen Section diagnosis: Negative for tumor by Touch Prep.

4) Sentinel node #4: 0.6 cm lymph node.

Frozen Section diagnosis: Negative for tumor by Touch Prep.

CONSULT: Mastectomy: 21.0 by 12.0 by 3.0 cm breast tissue with 13.0 by 5.5 cm skin ellipse, and blue dye at 12:00 position. Inked and sectioned revealing a 2.0 by 2.0 by 2.3 cm light-tan, well position under nipple. A second, 0.5 cm nodule is found inferior to (and 2.0 cm from) first nodule. Margins free of tumor.

FINAL DIAGNOSIS:

AFS) RIGHT SENTINEL LYMPH NODE #1:

- ONE (1) LYMPH NODE FOCALLY POSITIVE FOR METASTATIC CARCINOMA (SEE NOTE)

BFS) SENTINEL LYMPH NODE #2:

- ONE (1) LYMPH NODE NEGATIVE FOR TUMOR

C) RIGHT AXILLARY LYMPH NODE:

- ONE (1) LYMPH NODE NEGATIVE FOR TUMOR

D) RIGHT BREAST:

- INFILTRATING DUCTAL CARCINOMA, INTERMEDIATE NUCLEAR AND HISTOLOGIC GRADE (2.3 BY 2.0 BY 2.0 CM) WITH EXTENSIVE DUCTAL CARCINOMA IN-SITU, IMMEDIATE NUCLEAR GRADE WITH CENTRAL NECROSIS REPRESENTING 60% OF TUMOR VOLUME

- FAT NECROSIS, HEMORRHAGE AND GRANULATION TISSUE

- CONSISTENT WITH BIOPSY SITE AND RESIDUAL DUCTAL CARCINOMA IN-SITU

- INTRADUCTAL PAPILLOMA

- INTRAMAMMARY LYMPH NODE, NEGATIVE FOR TUMOR

- SKIN AND NIPPLE, NOT REMARKABLE

- MARGINS FREE OF TUMOR

E) RIGHT SENTINEL LYMPH NODE #3:

- NEGATIVE FOR TUMOR

F) RIGHT SENTINEL LYMPH NODE #4:

- NEGATIVE FOR TUMOR

NOTE: The metastatic focus is present in a vascular space in the capsule.

AE1/AE3 and EMA are positive. AE1/AE3 and EMA on the other sentinel lymph nodes are negative. ER/PR and HER-2/neu will be performed on block "D1".

## S U P P L E M E N T R E P O R T

(ER/PR)

My signature below is attestation that I have reviewed all slides and agree with the findings as noted below.

IMMUNOPEROXIDASE IDENTIFICATION OF ESTROGEN AND PROGESTERONE RECEPTORS IS CARRIED OUT ON SLIDE "D1". DISTINCT INTRANUCLEAR STAINING IS IDENTIFIED FOR ESTROGEN RECEPTOR (30%) AND PROGESTERONE RECEPTOR (<10%). THEREFORE, ESTROGEN RECEPTOR IS INTERPRETED AS BORDERLINE AND PROGESTERONE RECEPTOR AS. NEGATIVE.

PATHOLOGIST

## S U P P L E M E N T R E P O R T

(HER2/NEU)

My signature below is attestation that I have reviewed all slides and agree with the findings as noted below.

AS PER THE REQUEST OF DR. [REDACTED], c-erbB2 (HER-2/NEU) IMMUNOSTAINING IS CARRIED OUT ON PREVIOUS MAGEE SURGICAL [REDACTED] BLOCK "D1" (BREAST CANCER) USING A 1:300 DILUTION OF DAKO'S POLYCLONAL ANTIBODY A485 (DIRECTED AGAINST THE INTRACELLULAR DOMAIN OF c-erbB2) WITHOUT ANTIGEN RETRIEVAL. NO DISTINCT COMPLETE MEMBRANE STAINING IS IDENTIFIED. THEREFORE, c-erbB2 (HER-2/NEU) IS INTERPRETED AS NEGATIVE (SCORE 0).